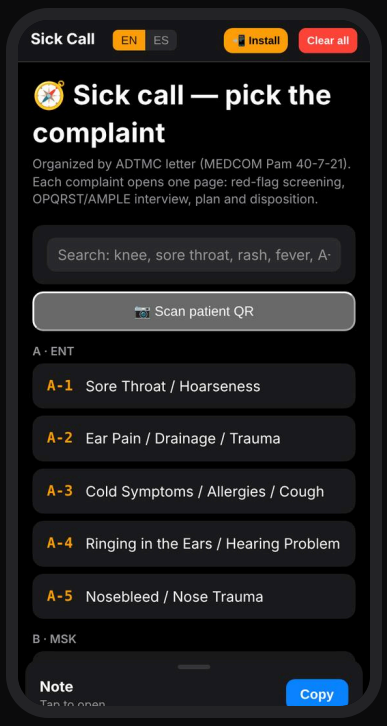
68W · ADTMC · MEDCOM PAM 40-7-21

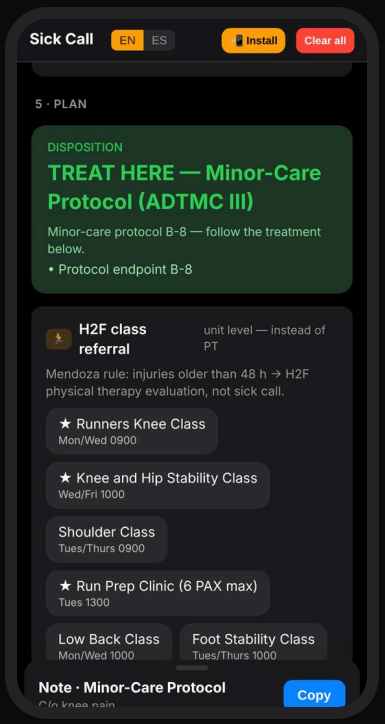
SICK CALL

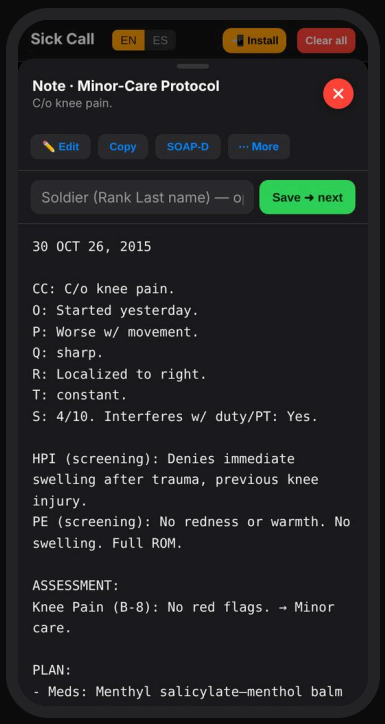
Operator's Manual

Complete guide to the sick call documentation app for 68W medics: installation, access, every feature step by step, troubleshooting and code architecture.

TM style | App v37 | Manual 1.1 | English







For trained medics only, under provider supervision. Not medical advice and not an official Department of the Army publication.

Important notices

WARNING **Not medical advice.** Sick Call is a documentation aid. It does not replace clinical judgment, the official ADTMC (MEDCOM Pam 40-7-21, 13 Nov 2019), local SOPs or a provider. Everything the app suggests is a **recommendation** under chain-of-command and provider supervision, not a medical order.

WARNING **For trained medical personnel only.** Not for self-diagnosis or self-treatment. A Soldier who feels sick uses the sick call questionnaire (Chapter 8) and sees their medic. Emergency: tell a medic now or call 911.

CAUTION The ADTMC content was transcribed by hand and may contain errors. Always verify against your official copy. Check with your chain of command before using any tool in an official workflow.

CAUTION **Never store real patient information** in the repository, screenshots, chats or this manual. Notes live only on the phone where they were written; delete them when sick call ends.

NOTE This manual follows the layout of an Army technical manual (TM) so it is easy to use in the unit, but it is **not an official publication** of the Department of the Army. All screenshots come from the real app (version v37) with **fictional data** (for example "SPC DOE, John A.", "SPC Lee"). The QR codes in the screenshots were made with a test key and do not work in the real app.

Application	**Sick Call — 68W ADTMC documentation tool**
App version	**v37 (APP_VER in app.js)**
Manual version	**1.1 — 30 October 2026**
Address	**https://bubymed.github.io/SICK-CALL-ADTMC-EASY-TOOL/**
Repository	**github.com/bubymed/SICK-CALL-ADTMC-EASY-TOOL**
App languages	**English and Spanish (the note is always written in English)**
Maintainer	**bubymed (repository owner)**

How to use this manual

Chapters 1–4 are for everyone: what the app is, how to install it, how to get access and how to move around the screen. Chapters 5–11 explain every feature step by step, in the order you would use them during sick call. Chapter 12 covers privacy, 13 troubleshooting and 14 the glossary. Chapter 15 is technical: it explains the code and architecture for whoever maintains the app.

Boxes follow TM conventions: WARNING = risk to the patient or to safety; CAUTION = risk of losing data or misusing the tool; NOTE = helpful information; TIP = shortcut or good practice.

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1 Introduction

Sick Call is a web app (PWA) that helps the 68W combat medic run an orderly sick call and write a short, clear note ready to transcribe to the slip. It follows the **ADTMC** algorithms (Algorithm-Directed Troop Medical Care, MEDCOM Pam 40-7-21).

What the app does

- **ADTMC protocols (86):** pick the complaint (knee, sore throat, fever...) and the app walks you through red flags, OPQRST/AMPLE history, decision points and plan, with the suggested disposition live.
- **Automatic note:** every tap writes the note in English, compact or SOAP-D (ADTMC Appendix D), ready to copy, print or move to the DD 689 / DA 5181.
- **Soldier QR check-in:** the Soldier answers about 1 minute of questions on their own phone and gets a QR; the medic scans it, the app sorts the line by priority and opens the protocol with the history already filled in.
- **Medications:** ADTMC Appendix C (44) and the pharmacy Self-Care OTC list, with “what is it for”, contraindications and daily maximums.
- **MACE 2:** concussion evaluation with a live result, PRA and a summary for the note.
- **Profiles, H2F and PFB:** profile recommendation builder (up to 14 days at unit level), H2F class referral with a printable slip, and the shaving profile (PFB).
- **Mendoza Clinic rules:** hours, requirements and what sick call is for; the QR triage logic applies them.
- **Unit QR access** for medics, with personal link, approvers (NCOs) and revocation.

Who it is for

User	What they can use
Approved medic (68W or another medical MOS)	The whole app: protocols, note, forms, saved patients, triage list, profiles, full MACE.
Approver (medic NCO)	All of the above + create unit QRs and approve medics of their unit.
App owner (admin)	Everything + approve approvers and revoke access.
Guest	Reference only: Meds, MACE (quick screen), Assessments, Questions, Mendoza Clinic.
Soldier	Only the sick call questionnaire (intake.html). They do not see the medic app.

Key facts

Item	Detail
Version	v37 (October 2026)
Address	https://bubymed.github.io/SICK-CALL-ADTMC-EASY-TOOL/
Works offline	Yes, after opening it once. Unit QR sign-up, the revocation list and updates need internet.
Where data is stored	Only in the phone browser (localStorage). There is no server or database.
Languages	Interface in English or Spanish. The note is always written in English.
Price	Free, no ads.
Clinical source	MEDCOM Pam 40-7-21 (ADTMC), 13 Nov 2019; local Mendoza Clinic (2/1 AD Soldier Care) rules.

What it does NOT do

- It does not diagnose: it suggests the ADTMC category; the medic and provider decide.
- It does not send notes to any system (it is not MHS GENESIS or AHLTA). The note is copied or printed.
- It does not store full names, DoD ID or Social Security numbers, and it warns you if you type them.
- The DD 689 and DA 5181 are fill-in replicas for printing convenience; the DD 689 “official PDF” uses the government template, but signatures and validity follow your SOP.

2 Installation and first start

Sick Call is not in the App Store or Google Play. You open it with a link and install it from the browser like any app. Once installed it opens full screen and works without signal.

Address: **https://bubymed.github.io/SICK-CALL-ADTMC-EASY-TOOL/**

Android (Chrome)

1. Open the link in **Chrome**.
2. Tap the orange **Install** button at the top right (or the ⋮ menu → **Install app** / **Add to Home screen**).
3. Confirm. From now on open **Sick Call** from the icon on your home screen.

iPhone (Safari)

1. Open the link in **Safari** (not inside WhatsApp or Instagram).
2. Tap **Share** (the square with the up arrow).
3. Choose **Add to Home Screen** and confirm.
4. Always open Sick Call **from that icon**.

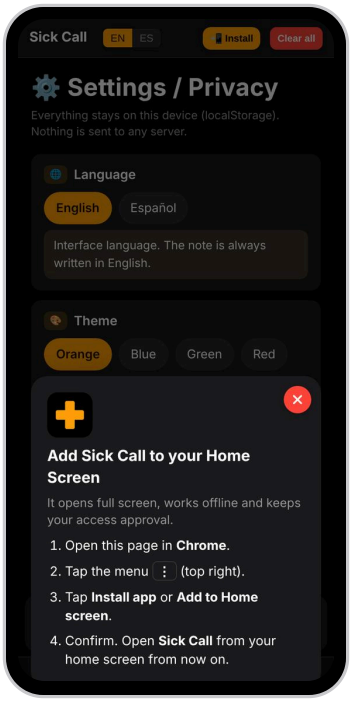


Figure 2-1. Install button on Android: Chrome steps.

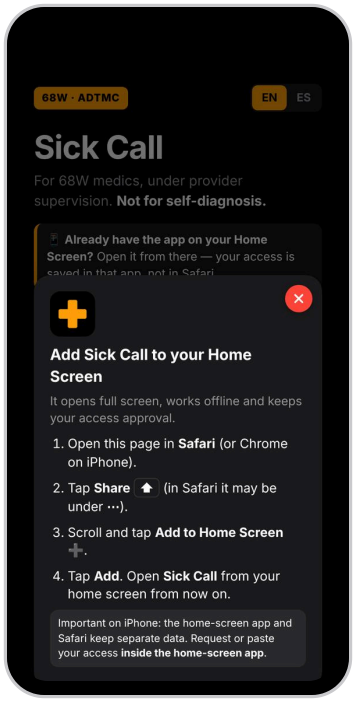


Figure 2-2. On iPhone the same button explains the Safari steps.

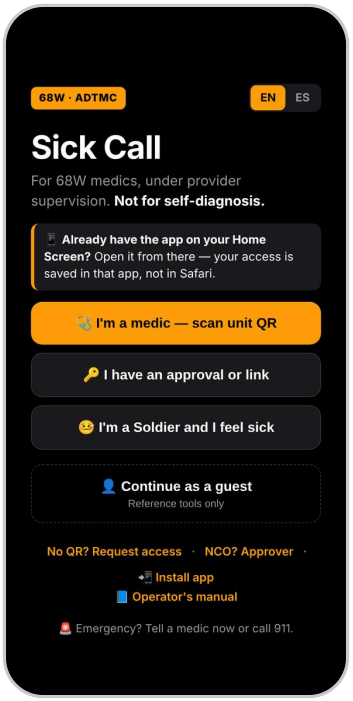


Figure 2-3. In Safari, the start screen reminds you to open the installed app.

WARNING **iPhone keeps Safari data and Home Screen app data separate.** If you sign up in Safari and then open the icon, the app asks for access again (and vice versa). Sign up inside the installed app, or use your **personal link** (Chapter 3) to move access from one to the other.

Start screen (first use)

The first time you open the app you see the start screen. At the top you can switch language (EN/ES). The options are:

Button	What it is for
I'm a medic — scan unit QR	Opens the camera to scan your unit QR and sign up (Chapter 3).
I have an approval or link	Paste or scan an approval, your personal link, an approver certificate or the admin key.
I'm a Soldier and I feel sick	Goes to the Soldier sick call questionnaire (Chapter 8).
Continue as a guest	Opens the reference tools only.
No QR? Request access · NCO? Approver	Create a request so an approver or the owner can approve you.
Install app	Shows how to install on your phone.
Operator's manual	Opens this manual (in guest mode).

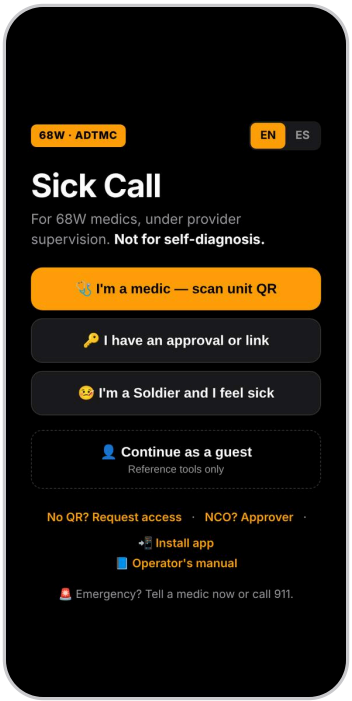


Figure 2-4. Start screen.

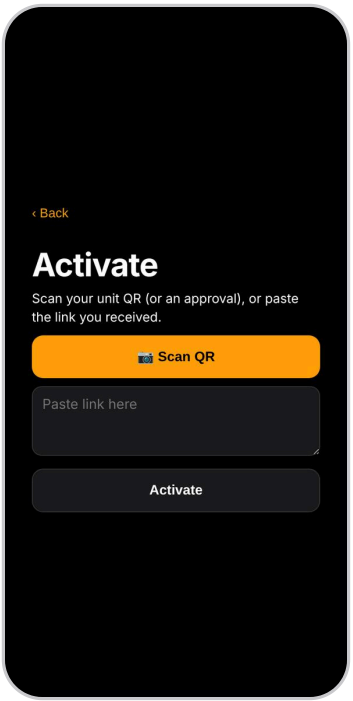


Figure 2-5. "I have an approval or link": scan or paste.

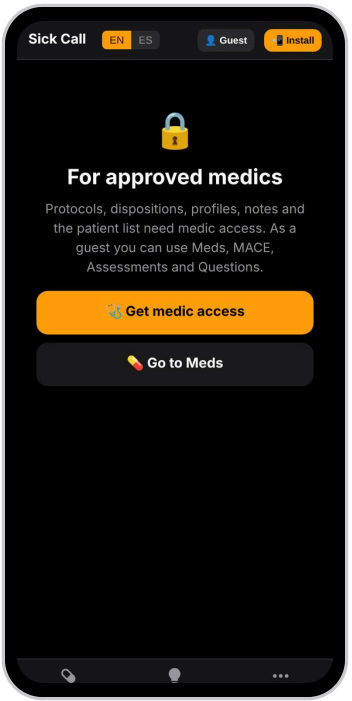


Figure 2-6. As a guest, medic features show as locked.

Updates

The app updates itself. Each time you open it or come back to it with internet, it checks for a new version; if there is one, it saves your work and reloads once. Then you see the **What's new** window with that version's changes. If something looks old, go to **More → Settings → Update app**: it clears the offline copy and loads the latest version without deleting your notes or settings.

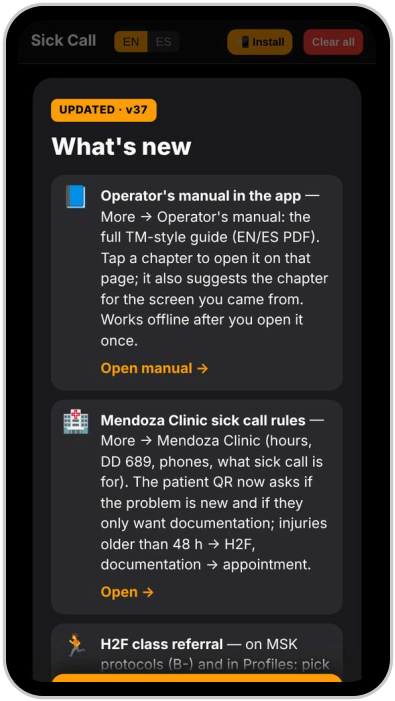


Figure 2-7. "What's new" window after an update.

3 Medic, approver and guest access

Clinical features (protocols, dispositions, notes, profiles and patient list) are for approved medical personnel only. Access is granted with digitally signed codes; there is no username or password and no server is needed.

Option 1 — Unit QR (easiest)

Your NCO or the app owner creates a QR for the unit. You scan it once:

1. On the start screen tap **I'm a medic — scan unit QR** (or scan it with the phone camera, or tap the link you were sent).
2. Fill in **rank, last name, first name + middle initial, MOS** (medical MOS list), **unit** and, if you want, a **point of contact** (optional).
3. Tap **Continue**, check your info and tap **Confirm & enter**. Needs internet.
4. Done: the phone now has medic access. Save your **personal link** (below).

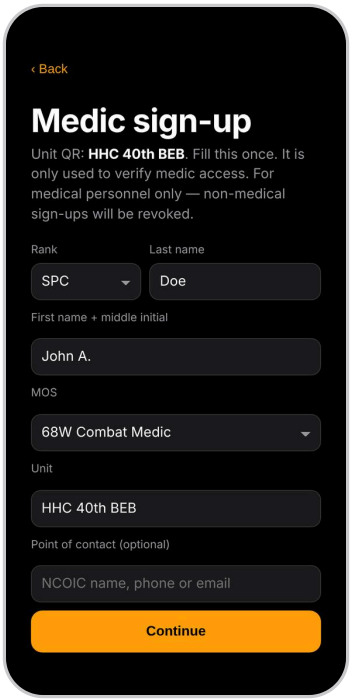


Figure 3-1. Medic sign-up from the unit QR.

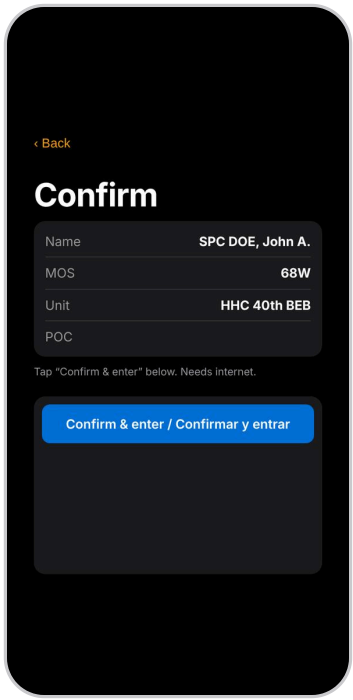


Figure 3-2. Confirm: the final button sends the sign-up.

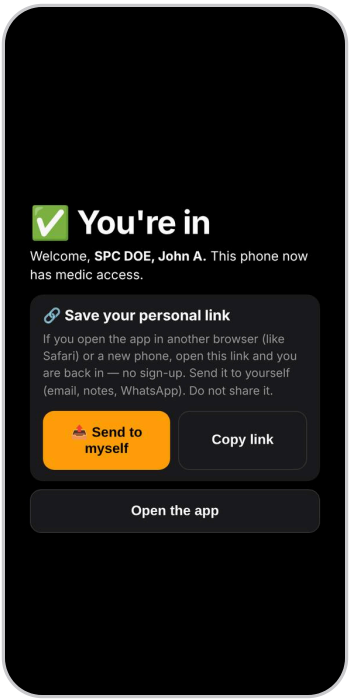


Figure 3-3. "You're in" and the box to save your personal link.

NOTE The sign-up is used only to verify that you are medical personnel. If you pick the last MOS option ("My MOS is not listed") the app does not let you continue and offers guest mode. Anyone who is not medical personnel and signs up will be revoked.

Your personal access link

The personal link gets you back in on another browser or a new phone without signing up again. You see it at the end of sign-up and in **More → Access → My personal access link**. Tap **Send to myself** or **Copy link** and keep it in your email, notes or a chat with yourself.

> **CAUTION** The personal link is **yours**. Do not share it: whoever opens it gets in with your access. If it leaks, the owner can revoke it by its ID.

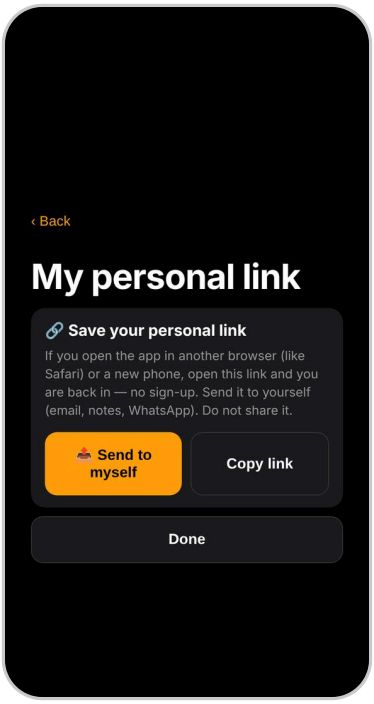


Figure 3-4. More → Access → My personal link.

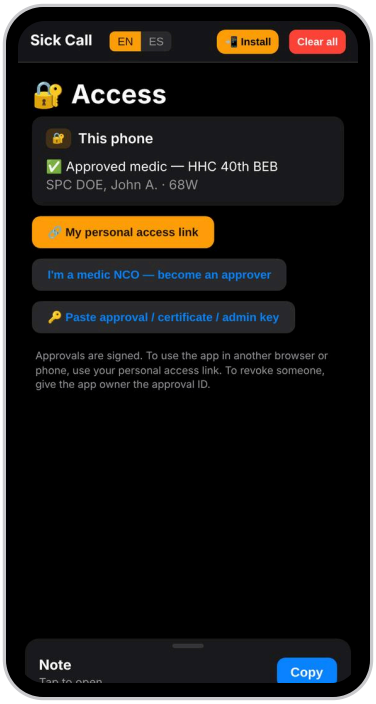


Figure 3-5. Access screen of an approved medic.

Option 2 — Request and approval (no QR)

1. On the start screen tap **No QR? Request access**. Type the unit and who you are (only your approver sees it).
2. Tap **Create request**. A QR and a link appear. In person, the approver scans it; remotely, share the link.
3. The approver opens it in **their** app (Access → Scan/Paste a request) and taps **Approve**. They get an approval (QR or link) to send back.
4. You go back to **I have an approval or link** and scan or paste it.

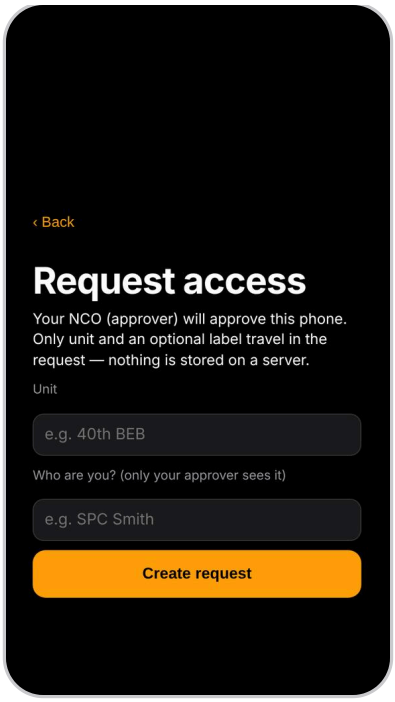


Figure 3-6. Request access without a unit QR.

CAUTION An approval works **only on the phone and app where the request was made**. If you open it in another browser, the app says “Open it in your app”. To change browsers, use your personal link.

Approvers (medic NCOs)

An approver can create unit QRs and approve the medics of **their** unit. To become one: **More → Access → I'm a medic NCO — become an approver** (or “NCO? Approver” on the start screen). The request goes to the app owner, who sends back an **approver certificate**; paste it in Access. Only the owner can approve approvers.

Create a unit QR

1. **More → Access → Create unit QR**.
2. Unit (an approver can only use theirs), **Valid for** 7, 14 or 30 days, and about how many medics.
3. Tap **Create QR**. A card appears with the steps, a red “medics only, do not forward” warning, the iPhone tip and the QR.
4. **Share** sends the card as an image + text + link (WhatsApp, Signal…). **Copy text** copies the message with the link.
5. Write down the **ID** shown below: it is used to revoke that QR.

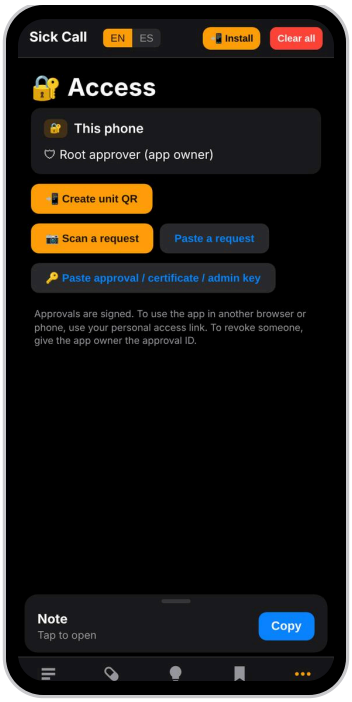


Figure 3-7. Access for an approver / owner.

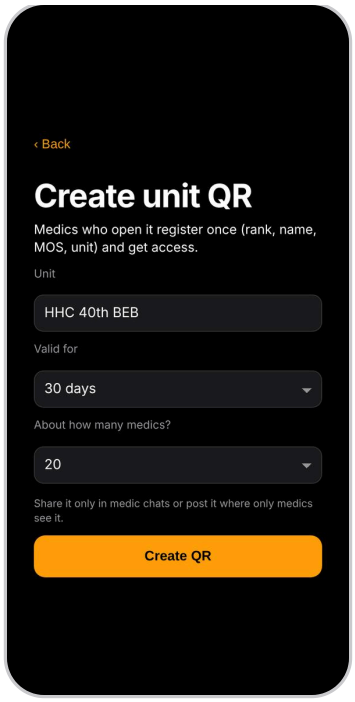


Figure 3-8. Create unit QR.

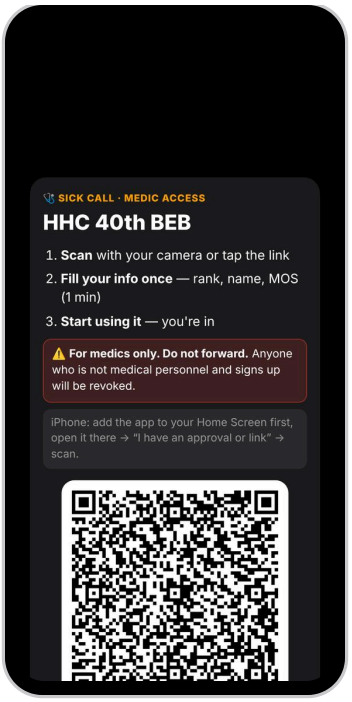


Figure 3-9. Instruction card ready to share.

> **WARNING** Share the unit QR only in medic chats or where only medics see it. You are responsible for who you approve.

Revoking access

Every approval, unit QR, personal link and phone has an ID. To remove access, give the ID to the app owner. The owner adds it to the repository revocation list (`revoked.json`); each phone checks it when the app opens with internet and, if its access is on the list, returns to the start screen. No new release is needed.

Guest mode

Continue as a guest allows the reference tools only: **Meds**, **MACE** (quick screen only), **Assessments**, **Questions** and the **Mendoza Clinic** page. Protocols, dispositions, profiles, notes and the patient list show a lock with a **Get medic access** button. The **Guest** button at the top also goes to the access screen.

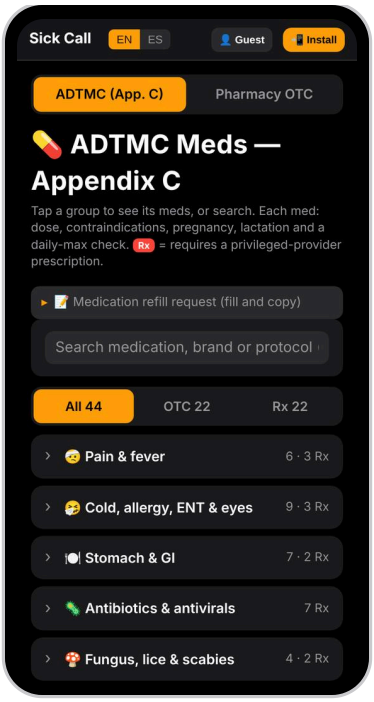


Figure 3-10. Meds in guest mode.

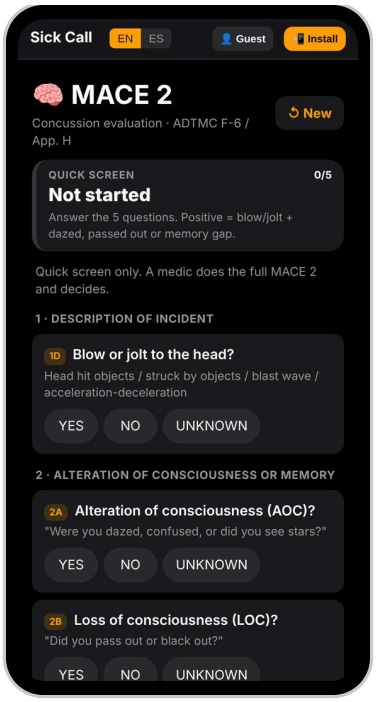


Figure 3-11. Guest MACE: quick screen only (1D and 2A–2D).

4 Screen tour

Every screen has the same structure: top bar, content in the middle, the note sheet at the bottom and the tab bar.

Part	What it does
Top bar	**EN / ES** switches language. **Install** appears if the app is not installed. **Clear all** clears the current case (asks to confirm) to start the next Soldier. On large screens there are also A−/A+ and "Clear section".
Tabs (bottom)	**Protocols** · **Meds** · **MACE** · **Saved** · **More**.
Note sheet	"Note" bar above the tabs: shows the disposition and summary; **Copy** copies the note without opening it. Tap it to open the full note (Chapter 6).
More	Operator's manual, What's new, Mendoza Clinic, Access, Survey, Scan QR, Triage list, QR poster, Demo, Questions, Assessments, Profiles, Settings, text size and clear.

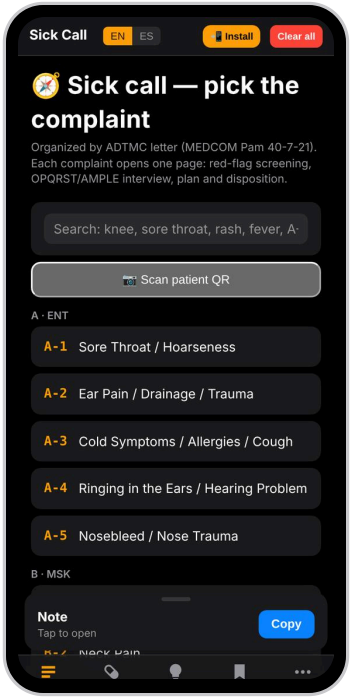


Figure 4-1. Protocols tab with the note bar and tabs.

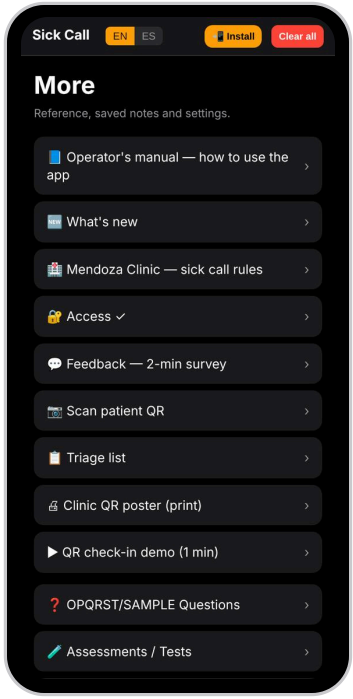


Figure 4-2. More tab.

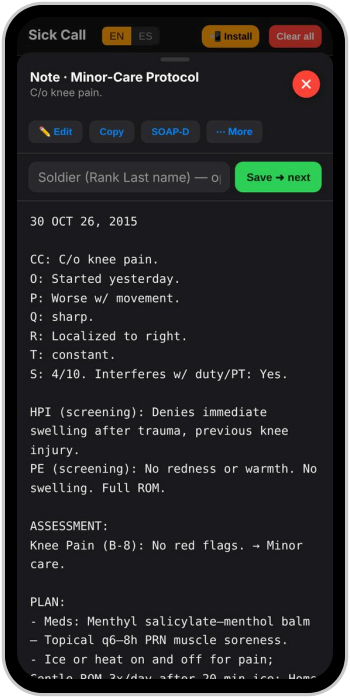


Figure 4-3. The note opened.

TIP On large screens (tablet or computer) the note stays fixed on the right while you work.

5 Seeing a patient: Protocols tab

This is the main workflow. Each ADTMC complaint opens a single page with everything in order: complaint, red flags, history, ADTMC decision points, plan and disposition. The note writes itself as you tap.

Step 1 — Pick the complaint

The 86 protocols are grouped by ADTMC letter (A · ENT, B · MSK, C · GI... M · Return). Search by typing at the top: “knee”, “sore throat”, “fever” or the code “B-8”. If you have patients from the QR you will see **Scan patient QR** and **Triage list**.

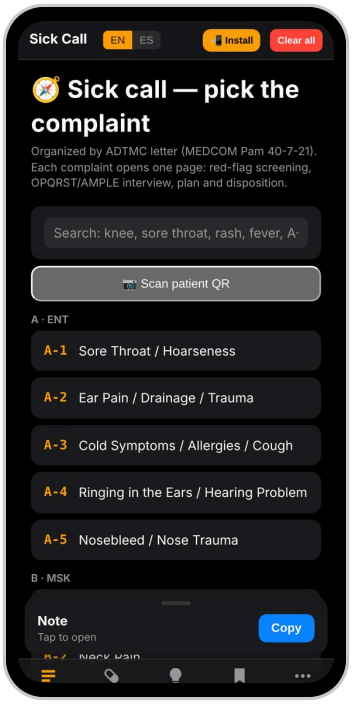


Figure 5-1. Protocol index.

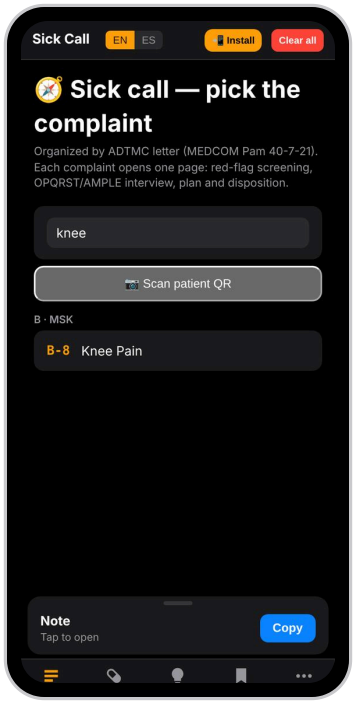


Figure 5-2. Search: “knee” → B-8.

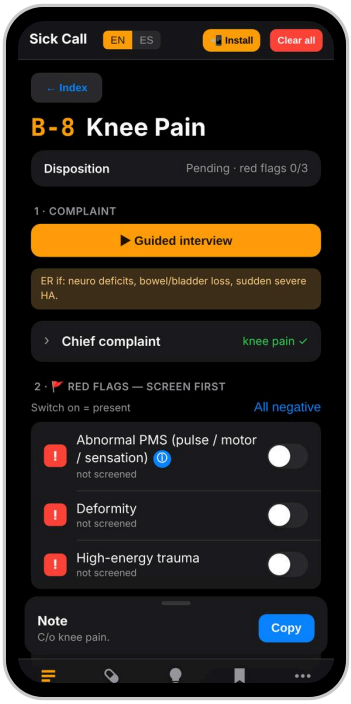


Figure 5-3. Protocol page B-8 Knee Pain.

Step 2 — Red flags first

At the top of the page is the live **Disposition** card (pending, in progress or the category). Then the protocol red flags. Each has a switch: **on = PRESENT**. If none is present, tap **All negative**. A positive red flag leads to **Provider Now** and blocks the RTD, Light Duty, H2F and Quarters dispositions; you can skip the rest of the history with “go to decision”.

The ⓘ button next to an exam (for example PMS) opens its “how to” card (Chapter 11).

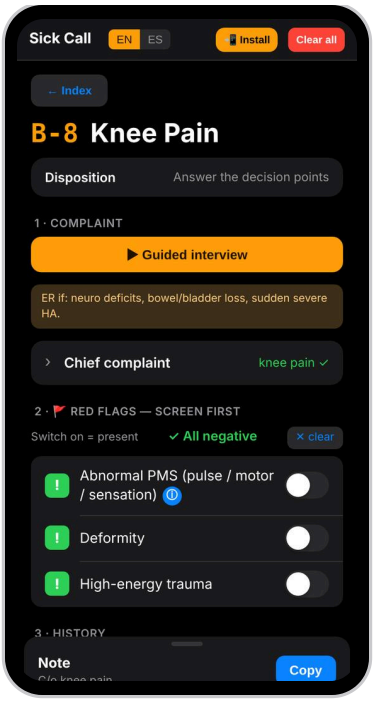


Figure 5-4. Red flags marked negative.

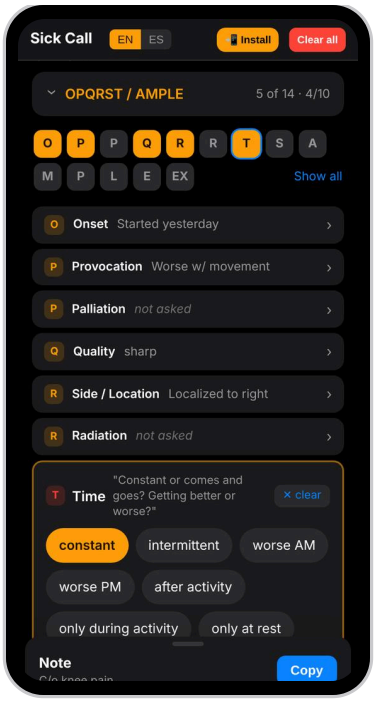


Figure 5-5. OPQRST/AMPLE history with folding letters.

WARNING If a red flag is present: do not recommend RTD, Light Duty, H2F or Quarters. Escalate to the provider/ER per protocol and notify your NCO or provider. Abnormal vital signs trigger the ADTMC global rule: **PROVIDER NOW**.

Step 3 — History (OPQRST / AMPLE)

The **History** section has one row per letter (O Onset, P Provocation, P Palliation, Q Quality, R Region, R Radiation, T Time, S Severity, and A M P L E). Tap options instead of typing; each letter has an "other / detail" field. Answered letters fold into a one-line summary and the letter bar at the top shows progress. Green options are specific to the complaint.

- **Severity** 0–10 and whether it **interferes with duty/PT**. At 8/10 or higher the app reminds you to review red flags and vitals.
- **Guided interview** (orange button): one question at a time, with Back / Next; "Next" with nothing selected skips the question.
- Some visits do not need a full history: the app shows "Reason for visit" or "Focused history" and a **+ Add full OPQRST / AMPLE** button.

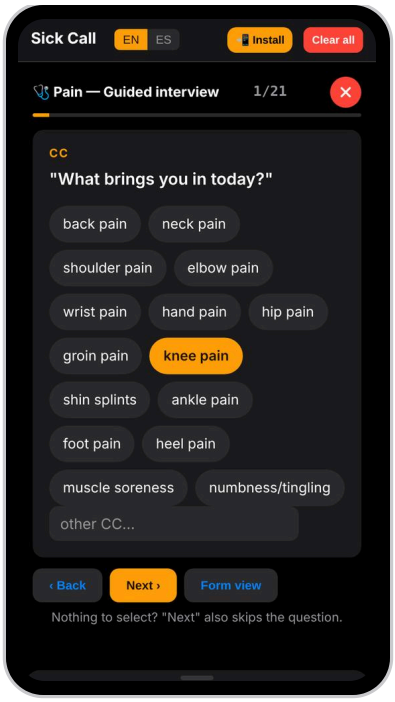


Figure 5-6. Guided interview: one question at a time.

Step 4 — ADTMC decision points

Each protocol has its decision points copied from the algorithm. Tap what applies or **None / next**. The app follows the flowchart: a "yes" can lead to Provider Now or AEM Now, ask for a test (for example a rapid strep test) or send you to another protocol. The **Differential** section lists the ADTMC possibilities. In **Vitals · LMP · return visit** (optional) you record vital signs, orthostatics, female data (LMP, pregnancy, lactation) and MRC status.

Step 5 — Decision, plan and disposition

When the decision points are done, the **Disposition** box shows the ADTMC category and why, and below it the suggested plan: protocol meds, care, activity and when to return. The card at the top of the page shows the disposition too.

Category	Meaning in the app
I — Provider Now	Refer to the clinic now, to the provider. Red flags or abnormal vitals.
II — AEM Now	See the AEM now. If your unit has no AEM (Settings), "AEM Now" goes to the clinic.
III — Minor-Care Protocol (MCP)	Treat here with the minor-care protocol and the ADTMC plan.
IV — Specialty referral	Refer to the service named by the protocol.
Appointment / referral	Schedule a clinic appointment (for example, documentation only or a chronic problem).

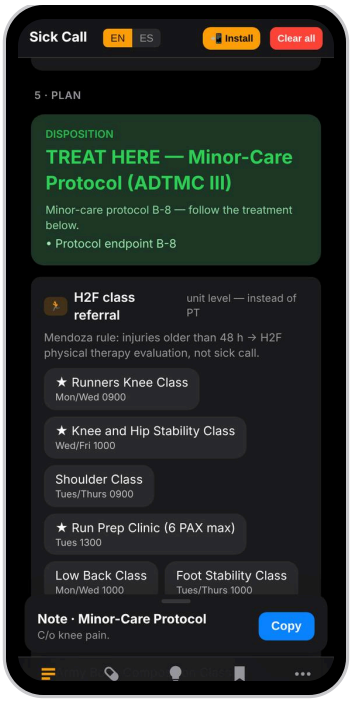


Figure 5-7. Disposition: treat here (MCP III).

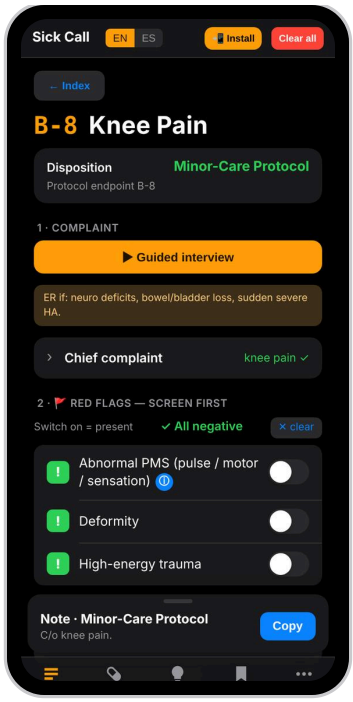


Figure 5-8. The top card sums up the live disposition.

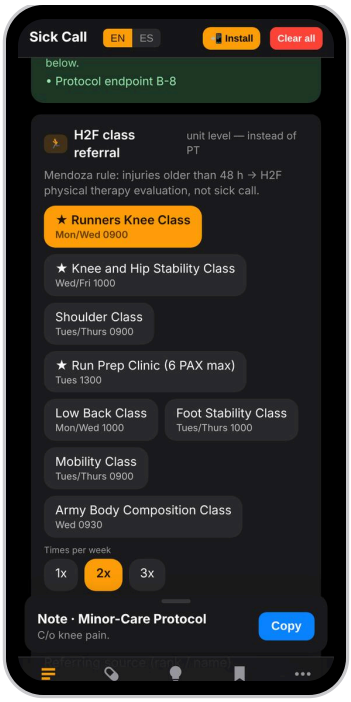


Figure 5-9. H2F class referral on a B- protocol.

H2F class referral (B- protocols)

On musculoskeletal protocols you see **H2F class referral**, a unit-level option instead of asking for physical therapy. Classes with ★ are suggested for that complaint. Pick one or more classes, **times per week** (1x, 2x, 3x), **weeks** (2, 4, 6, 8) and the referring source. The note line writes itself and you can print the H2F slip (Chapter 6). Mendoza rule: an injury older than 48 h goes to H2F physical therapy evaluation, not sick call.

Shaving profile (PFB, J-3)

On J-3 Shaving Problem you get the **Shaving profile (PFB)** card: pick Mild, Moderate or Severe and the app gives the phase, plan and e-Profile wording. At the top are the Provider Now signs (cellulitis, abscess, scarring, needing limits on protective equipment).

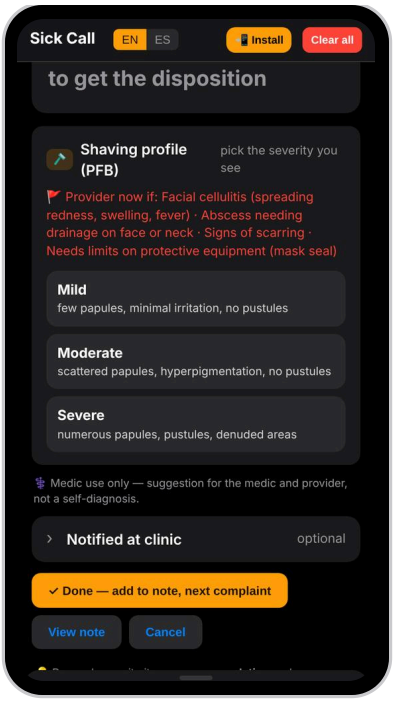


Figure 5-10. PFB card on J-3.

Step 6 — Finish or add another complaint

Tap **Done — add to note, next complaint** to keep this protocol in the note and go back to the index: you can see another complaint for the same Soldier and everything combines in the same note. **View note** opens the note; **Cancel** discards this protocol. If the disposition is a clinic referral you will see a line with the Mendoza sick call hours.

TIP Write everything as a **recommendation** under chain-of-command and provider supervision, not as a medical order. For meds: “to be picked up at Mendoza Pharmacy”.

6 The note and forms

The note builds itself in English from what you tap. Open it by tapping the “Note” bar at the bottom.

Button	What it does
Edit	Lets you change the note by hand. While editing, the note no longer rebuilds; ↻ **Auto** discards manual edits and rebuilds it.
Copy	Copies the whole note to the clipboard.
SOAP-D	Switches between compact and SOAP-D (Subjective, Objective, Assessment, Plan, Disposition — ADTMC Appendix D).
More → Download .txt	Downloads the note as a text file.
More → Print note	Print or save as PDF.
More → DA Form 5181	Opens the DA 5181 (Screening Note of Acute Medical Care) replica filled from the note.
More → DD Form 689	Opens the DD 689 (Individual Sick Slip) filled in; includes “Official PDF”.
More → H2F slip	Opens the H2F Class Appointment Slip with the classes marked.
Soldier (Rank Last name) — optional	Short label to find the note in Saved. Rank and last name only.
Save ➔ next	Saves the note in Saved and clears everything for the next Soldier.

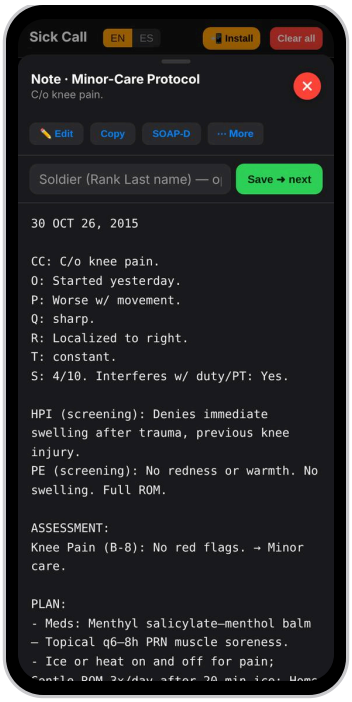


Figure 6-1. Note in compact format.

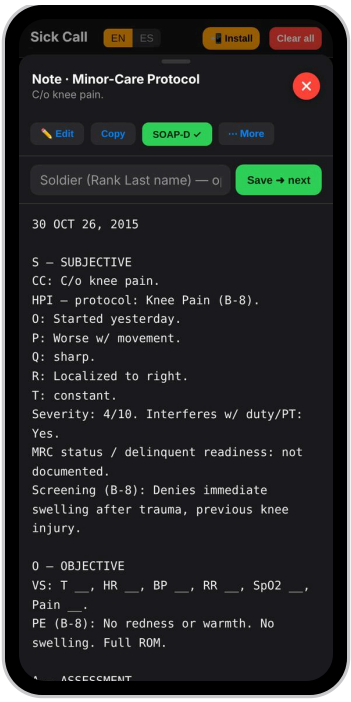


Figure 6-2. The same note in SOAP-D.

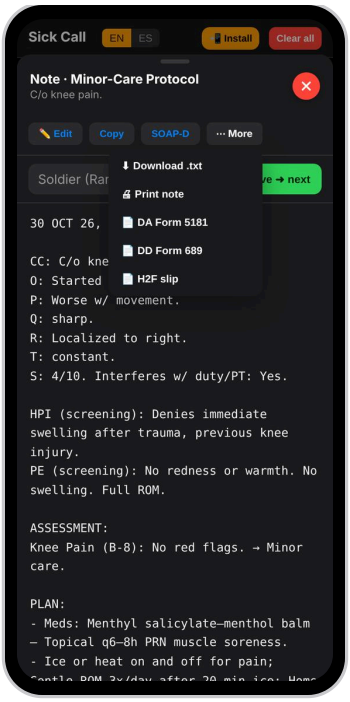


Figure 6-3. Note More menu.

> **CAUTION** The app detects personal information (SSN, DoD ID, date of birth, phone, email, medical record number) and shows a "Personal info detected" warning. Remove it before saving. **Never store real patient identifiers.**

DD Form 689 — Individual Sick Slip

The DD 689 is filled with the complaint, date and note summary. Buttons: **History below: ON/OFF** (adds or removes the history in remarks), **Official PDF** and **Print**. Name, DoD ID, rank and signature are **not saved**: write them by hand or at print time. Tick blocks 10–11 and sign with your finger if your SOP authorizes it. Blocks 7–9 stay blank for the Commander.

The **Official PDF** fills the government template (DD 689, AUG 2017) on the phone, **flattens** the fields (no longer editable) and adds a footer: "Generated … by Sick Call (flattened copy). Any alteration voids this document."

> **NOTE** For real tamper protection, sign the PDF with your CAC in Adobe. Flattening prevents casual edits; it is not a digital signature.

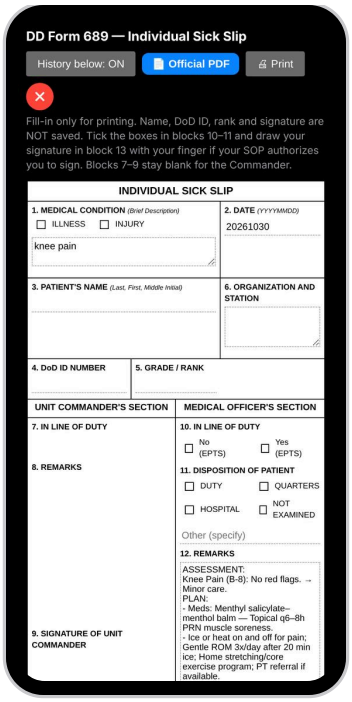


Figure 6-4. DD 689 filled from the note.

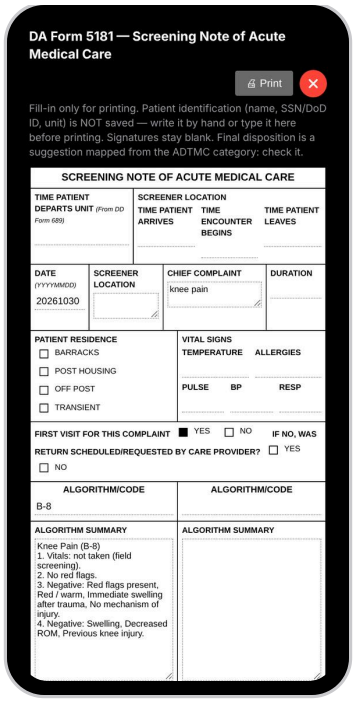


Figure 6-5. DA 5181 replica.

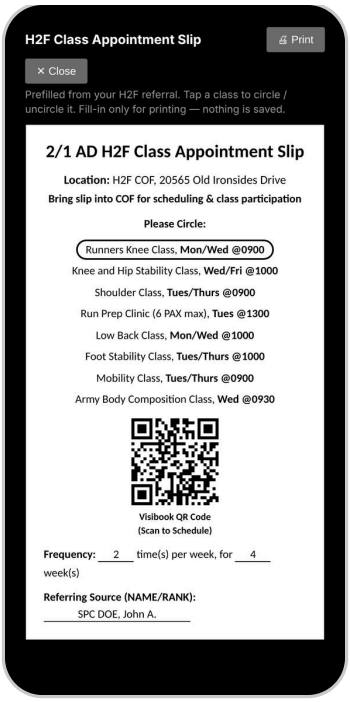


Figure 6-6. H2F class slip, ready to print.

DA Form 5181 and H2F slip

The DA 5181 is a fill-in replica for printing; patient identification is **not saved** (write it by hand). The final disposition is mapped from the ADTMC category as a suggestion: check it. The H2F slip has the location (H2F COF, 20565 Old Ironsides Drive), the classes and times, the Visibook QR to schedule, frequency, weeks and referring source. It goes to H2F, not to the provider.

7 Saved patients

The **Saved** tab keeps the notes you finished with "Save ➔ next", in this browser only.

- Each note shows rank and last name, the time and the text.
- **follow-up** reopens the patient as a return visit (for M-1 "not getting better" or M-2 "return requested by provider").
- **copy** copies the note; **delete** removes it; 🗑️ **Delete all** removes them all.
- After the first saved note the app asks (only once) for 2 minutes for a feedback survey.

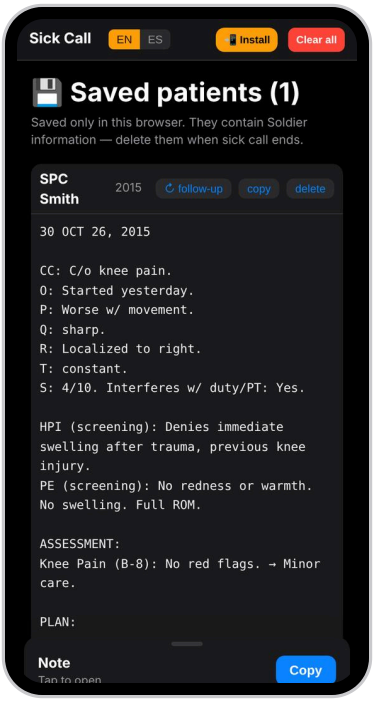


Figure 7-1. Saved patients.

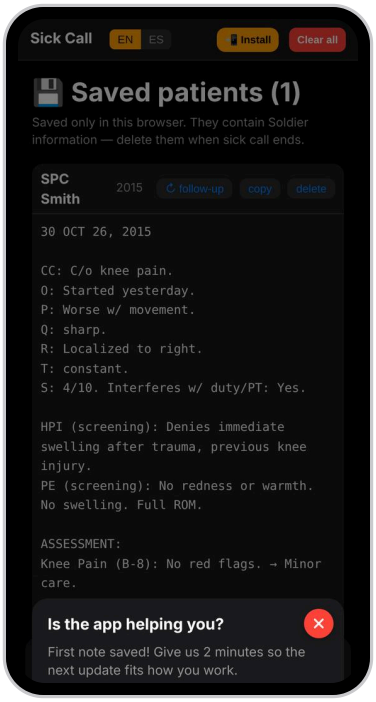


Figure 7-2. Survey invitation after the first note.

CAUTION Saved notes contain Soldier information. Delete them when sick call ends. In Settings you can turn on **daily auto-delete** and export a backup (.json) of what you need first.

8 Soldier QR check-in and triage list

The Soldier scans the sick call poster and answers about 1 minute of questions on their own phone. At the end they get a QR that helps the medic prioritize the line at a glance. The medic scans it and the app opens the right protocol with the history already in the note.

Soldier side (intake.html)

1. **Start.** Warning: trouble breathing, chest pain, heavy bleeding or passed out → do not fill this out; tell the medic or call 911.
2. **Before you start:** “Be honest” and the UCMJ Article 115 notice (malingering). The button unlocks after a few seconds.
3. **Rank and last name** — nothing else (no first name, no DoD ID).
4. **What's bothering you?** Pick the main problem by group.
5. **A few details** for that problem (for example: what happened, is it swollen, can you bear weight).
6. **When did it start?** Today · 1–2 days · 3–7 days · 1–4 weeks · more than a month.
7. **Had it before?** · **Worse in the last 2–3 days?** · **Already seen for this?** (and worse since) · **Here only for documentation?**
8. **How bad?** 0–10 · **Does it stop your job or PT?**
9. **Warning signs** (trouble breathing, chest pain, fainting, bleeding that won't stop, very high fever…) or “None of these”.
10. **Anything else** (optional, 100 characters, no names or ID numbers) → **Get my code**.

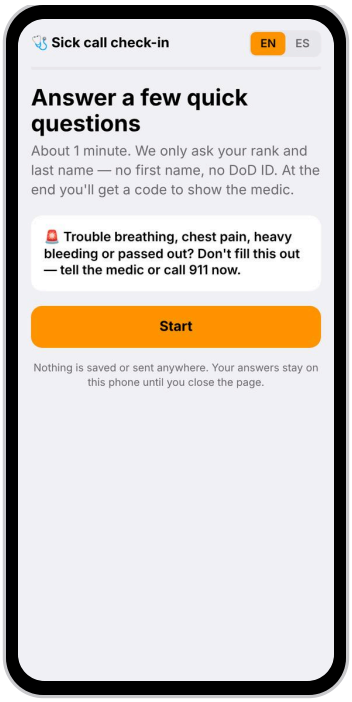


Figure 8-1. Questionnaire start.

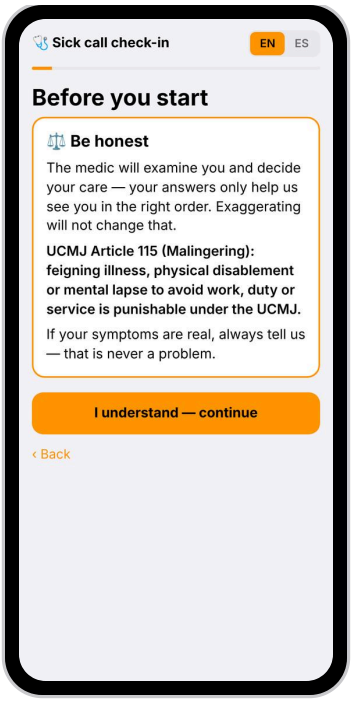


Figure 8-2. Rules: be honest and UCMJ Art. 115.

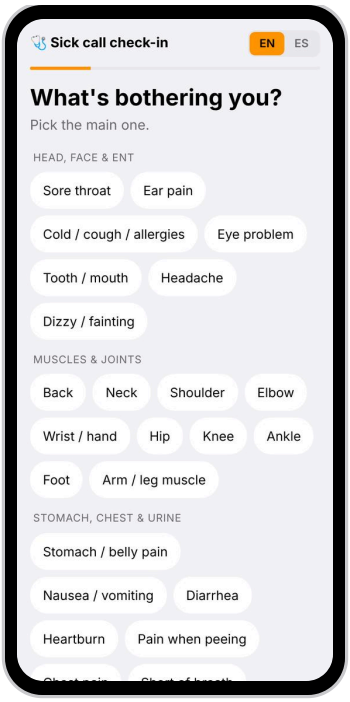


Figure 8-3. What's bothering you?

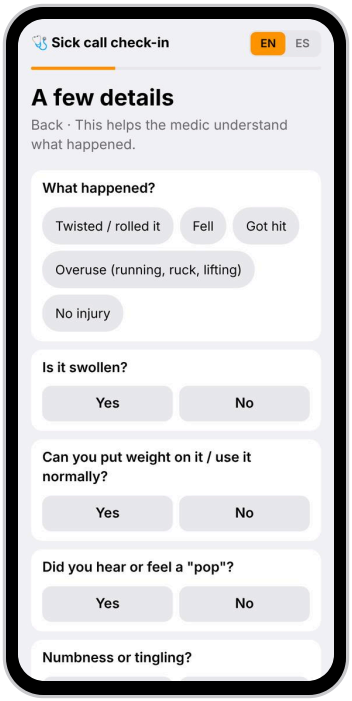


Figure 8-4. Injury details.

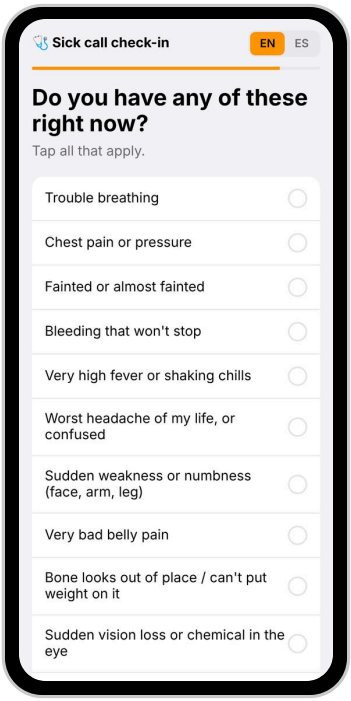


Figure 8-5. Warning signs.

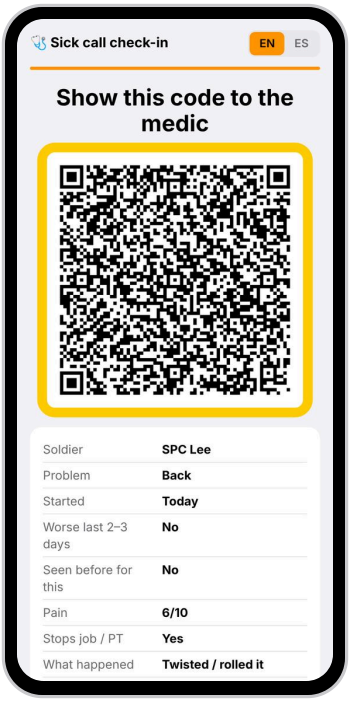


Figure 8-6. The code to show the medic.

The final screen shows the QR with a colored frame by triage level, a summary of the answers and the Mendoza Clinic box (hours, DD 689, what sick call is for and appointment phones). **Nothing is saved or sent**: the answers live on the page until it is closed. If they cannot come in person, they take a screenshot and send it to their medic.

WARNING If the Soldier marks a warning sign, the screen says “Tell the medic NOW — don't wait in line.” If they mark thoughts of self-harm it shows the Military/Veterans Crisis Line: call 988, then press 1 (24/7).

How triage is decided (Mendoza rules)

The app decides the color in this order; the first rule that applies wins:

#	Rule	Color
1	Any warning sign	Red — See now
2	Administrative request	Gray
3	Documentation only (chronic / VA)	Gray — appointment
4	Shaving (PFB)	Gray
5	Return visit for the same problem	depends on whether it is worse
6	Musculoskeletal injury: under 48 h → orange; over 48 h → gray with “H2F PT evaluation”	Orange / Gray
7	Active fever, nausea, vomiting or diarrhea	Orange — Sick call
8	Old problem that is not worse	Gray — appointment
9	Started within the last 48 h	Orange
10	Worse, or severity 8/10 or more	Orange
11	Everything else	Gray — consider appointment

Medic side: scanning and the triage list

1. Tap **Scan patient QR** (Protocols index or More). Point the camera at the code. If the camera is blocked, use **Choose a photo or screenshot**, or scan with the iPhone Camera app: it opens Sick Call with the answers.
2. The patient goes to the **Triage list**, sorted: **See now** (red), **Sick call / screening** (orange) and **Consider appointment** (gray), with the rule that put them there, onset, pain and whether it stops duty.
3. Tap **Start A-1 ›** (or the matching protocol): it opens with onset, severity, duty impact and return visit already filled, plus a banner of what the patient reported. **Check them.**
4. **Remove** takes a patient off; **Clear list** empties it. The list deletes itself after 12 hours and stores no full names or IDs.

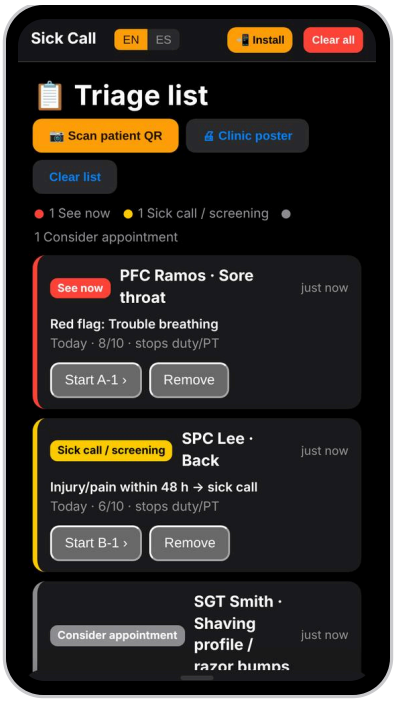


Figure 8-7. Triage list with three fictional patients.

More → Clinic QR poster (print) opens the poster to put up at the clinic or sick call area. **More → QR check-in demo** plays a 1-minute video.

9 Meds tab

Medication reference with two sources you switch at the top: **ADTMC (App. C)** and **Pharmacy OTC**.

ADTMC — Appendix C

- 44 medications grouped by use: Pain & fever; Cold, allergy, ENT & eyes; Stomach & GI; Antibiotics & antivirals; Fungus, lice & scabies; Skin, wounds & acne...
- **All / OTC / Rx** filter. **Rx** = requires a privileged provider prescription.
- Each med: dose, protocols where it appears, ADTMC page and a **contraindications / pregnancy / lactation** drop-down with a daily-max check.
- The ⓘ button explains **what it is for**: what it does, what it is best for and when to pick another.
- Search by medication, brand or protocol.
- **Medication refill request** (fill and copy): builds a message to request a refill. It sends nothing and stores no data.

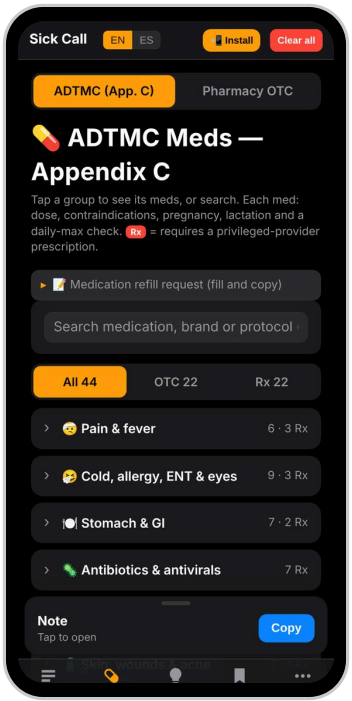


Figure 9-1. Meds: Appendix C by groups.

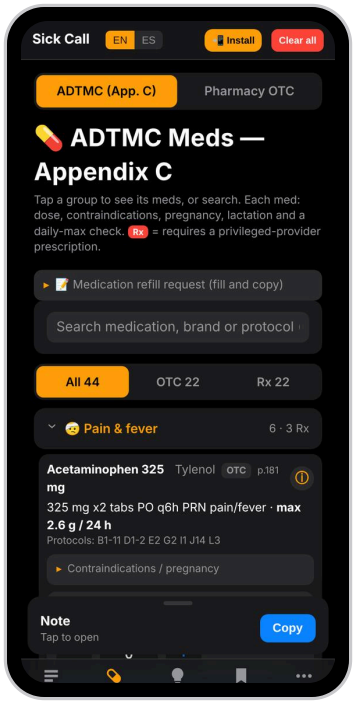


Figure 9-2. Medication card open.

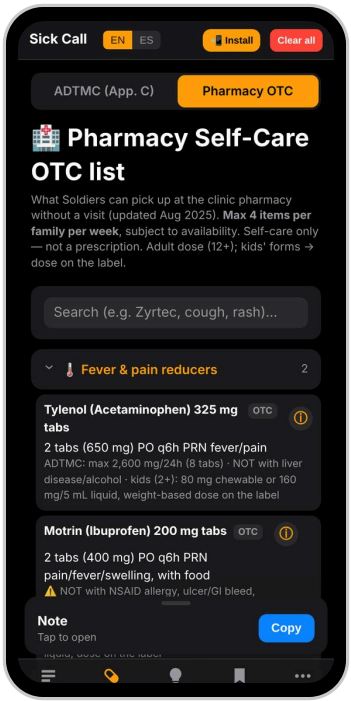


Figure 9-3. Pharmacy OTC list.

Pharmacy Self-Care OTC list

What Soldiers can pick up at the clinic pharmacy without a visit (updated August 2025): **max 4 items per family per week**, subject to availability. It is self-care, not a prescription. Adult dose (12+); kids' forms → dose on the label.

NOTE Symptoms over 1 week despite OTC → provider. Red flags (fever >103, SOB, chest pain, neuro deficit, uncontrolled bleeding) → ER. More than 4 items/week or a med not on the list → provider. Document as a **self-care recommendation**.

10 MACE tab (MACE 2)

Concussion (mTBI) evaluation per ADTMC F-6 and Appendix H. The status card stays pinned at the top while you answer.

Approved medic

- **1–4 · Screening:** description of incident (1D, blow or jolt to the head) and alteration of consciousness or memory (2A AOC, 2B LOC, 2C PTA). Positive = blow/jolt + AOC, LOC or PTA.
- **Cognitive exam** (word and digit lists), **neuro exam + VOMS** and symptoms (NSI) on one line per symptom.
- **Summary → note**: sends the result to the note.
- **Return to activity (PRA)** by stages and **After MACE: coding & next steps**.
- Negative screen: stop MACE 2, 24 h rest, follow the CMT. The ↻ **New** button starts another evaluation.

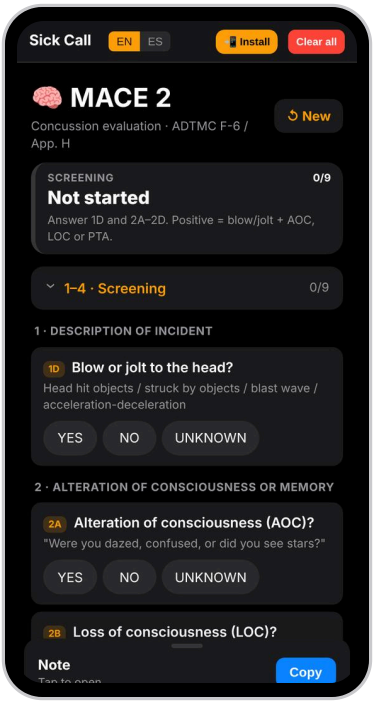


Figure 10-1. MACE 2 with the pinned status card.

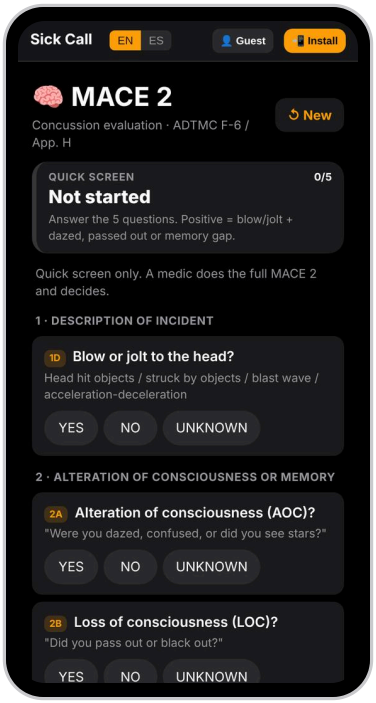


Figure 10-2. Guest version: quick screen only.

Guest or Soldier

They answer only 1D and 2A–2D and see **Positive / Negative**. Positive: “Possible concussion. Go to a medic or aid station NOW. No driving, no PT, no weapons until seen.” It does not diagnose or create notes: the medic does the full MACE 2.

CAUTION The MACE 2 word and digit lists were transcribed from the pamphlet; compare them with your printed form.

11 More: references, profiles and settings

Operator's manual (in the app)

More → Operator's manual (also on the start screen and in guest mode) opens this manual inside the app. Pick **English** or **Español**, tap **Open the full manual**, or tap a chapter to open the PDF on that page. The blue box suggests the chapter for the screen you came from (for example, from Meds it points to Chapter 9). After you open it once with internet, it also works offline.

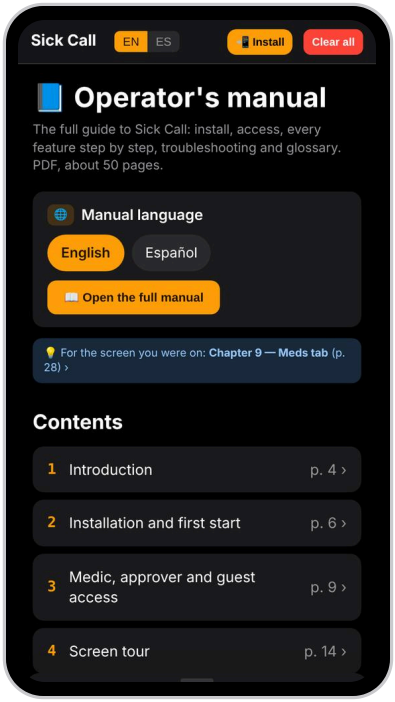


Figure 11-1. More → Operator's manual, opened from the Meds tab.

NOTE On iPhone the PDF viewer may open on page 1; scroll to the page number shown next to the chapter.

Mendoza Clinic — sick call rules

Mendoza Clinic (2/1 AD Soldier Care), 11335 SSG Sims St, Fort Bliss, TX 79918. Appointments: (915) 742-2273 · (915) 742-0186. **Sick call 0630–0700, Monday–Friday**: Soldiers sign in on the clipboard at the front desk and a **DD 689** is required. No DD 689 but meets criteria: may be seen by the provider on duty 0830–1600 (wait may exceed 2–3 h). The medic and provider determine what is medically appropriate.

- Sick call is **only** for: active fever, nausea, vomiting or diarrhea; new-onset pain, rash or new symptoms never had before; an injury within the last 48 hours.
- Injury older than 48 h → H2F physical therapy evaluation.

- "I just want it documented" (chronic / VA) → appointment by phone.

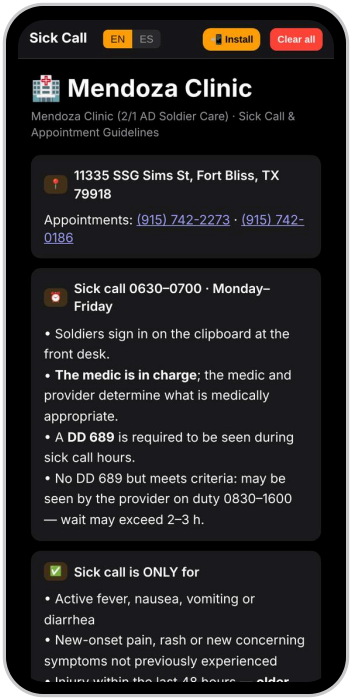


Figure 11-2. More → Mendoza Clinic.

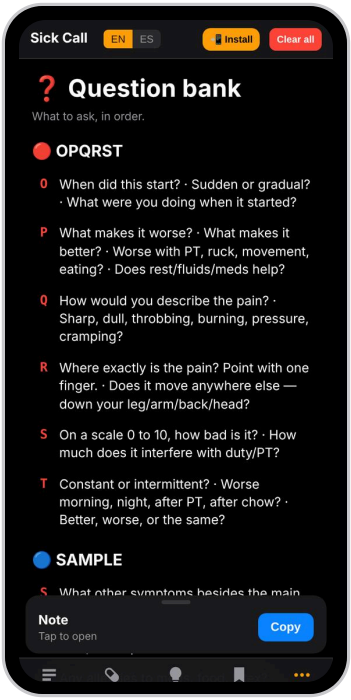


Figure 11-3. OPQRST / SAMPLE question bank.

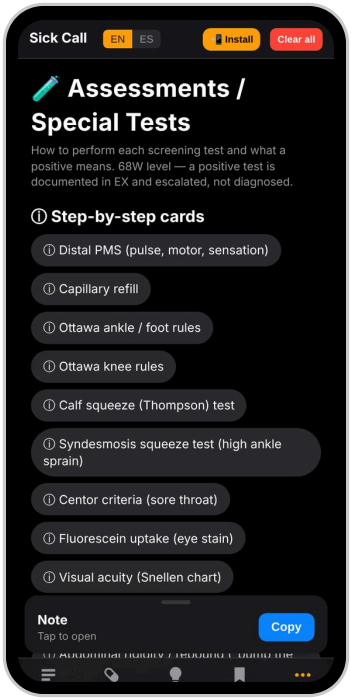


Figure 11-4. Assessments / special tests.

Questions and Assessments

OPQRST/SAMPLE Questions is the question bank in order, so you ask without forgetting anything. **Assessments / Tests** explains how to perform each screening test and what a positive means at 68W level: a positive is documented in the exam and escalated, not diagnosed. The **step-by-step cards** (distal PMS, capillary refill, Ottawa ankle and knee rules, Thompson, squeeze, Centor, fluorescein, visual acuity, orthostatics, rebound, Battle sign, stiff neck, neuro check, pupils, SIRS, range of motion) say what it is, how to do it, what is normal, what is positive and what to do.

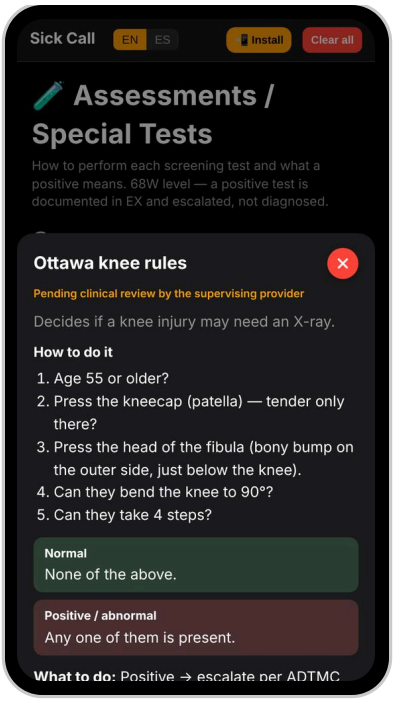


Figure 11-5. Step-by-step card: Ottawa knee rules.

> **NOTE** Cards show “Pending clinical review by the supervising provider” until a provider reviews them. If you do not know how to do a test, mark it as not done and ask your NCO, AEM or provider; never guess.

Profiles — builder

Builds the profile recommendation: **Type** (RTD full duty, RTD w/ light duty, Quarters), **Duration** (24h, 48h, 72h, 7 days, 14 days), **Restrictions** (no PT, no ruck, no running, no jumping, no lifting >20 lbs, no prolonged standing >30 min…), **Follow-up / Referral** and **Shortcuts** with common combinations. The text generates itself: copy it or send it to the note disposition. If you type personal info, the text is blocked.

- **Unit level:** temporary profiles up to 14 days, H2F referral and basic treatment. Re-evaluate at the end of the profile.
- Not improved → clinic (Mendoza). PT referrals and longer or permanent profiles are done by the provider at the clinic.
- The shaving profile (PFB) and H2F referral are here too.

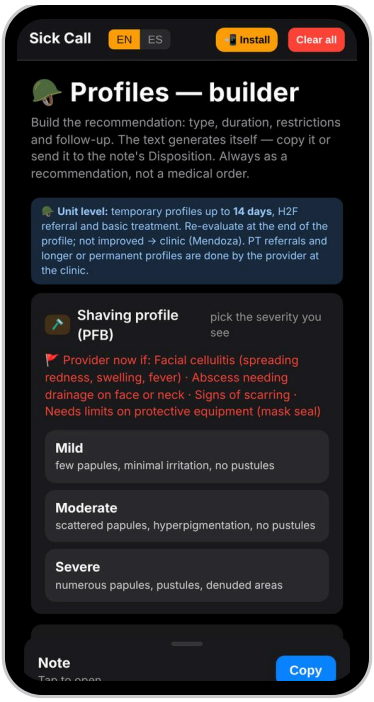


Figure 11-6. Profile builder.

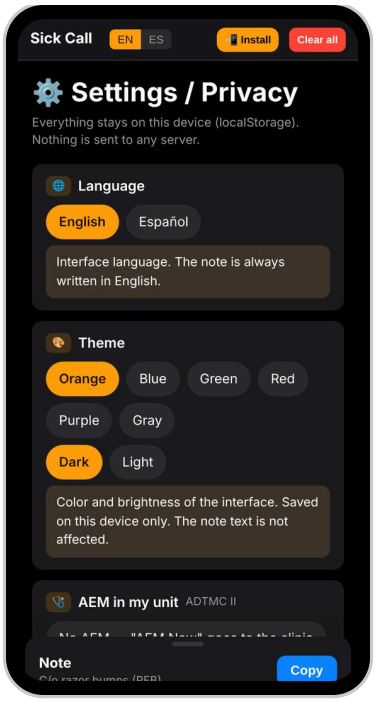


Figure 11-7. Settings / Privacy.

Settings / Privacy

Setting	What it does
Language	English / Español. Only the interface; the note is always in English.
Theme	Color (orange, blue, green, red, purple, gray) and dark/light. Does not change the note.
AEM in my unit	With AEM available, “AEM Now” goes to the AEM; without AEM, to the clinic.
Note detail (screening)	Full (every pertinent negative), Short (skip the red-flag “Denies…” list) or Minimal (positives only).
Folded sections	Each protocol section is one line that opens on tap, or everything open (classic).
Compact history	Answered letters fold up as you move on.
Daily auto-delete	On open, deletes notes and data from previous days. Recommended.
Lock PIN	Hides the screen in this browser. **It does not encrypt data.** If you forget it, “Delete everything” removes it.
Backup	Export saved (.json) and import.
App version / Update app	Shows the version and reloads the latest without deleting notes or settings.
Delete EVERYTHING on this device	Deletes notes, settings and app data in this browser.

Setting	What it does
Source and scope	Clinical source and known limits.

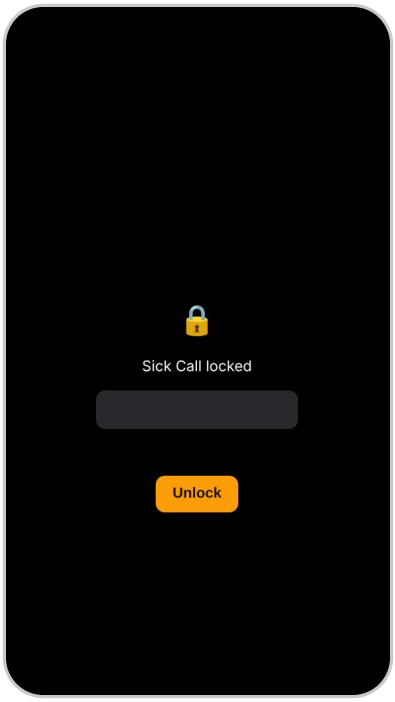


Figure 11-8. Screen locked with a PIN.

Other More options

- **What's new**: shows the latest changes again.
- **Feedback — 2-min survey**: opens the survey to improve the app.
- **Text size** A− / A+ and **Clear section / Clear all**.

12 Privacy and security

- Everything runs in your browser. Notes, saved patients, the triage list and settings live in the phone local storage (localStorage); **they are not sent to any server**.
- The Soldier questionnaire saves and sends nothing: the information travels only inside the QR the medic scans.
- Medic sign-up through a unit QR sends rank, name, MOS, unit and the optional point of contact to a sign-up form (Tally) to verify access. It contains no patient information.
- When it opens with internet, the app downloads `version.json` (updates) and `revoked.json` (revocations). It sends none of your data.
- The PIN only hides the screen; it does not encrypt. Protect your phone with its own lock.
- The personal-info detector warns you if you type an SSN, DoD ID, date of birth, phone, email or medical record number.

WARNING Never upload real notes to the repository or paste them in chats or screenshots. Always use fictional data for tests, demos and manuals.

13 Troubleshooting

Problem	Likely cause	What to do
iPhone asks for access again	You opened Safari instead of the installed app (they keep separate data).	Open the Home Screen icon, or open your personal link in that browser.
“QR expired”	The unit QR passed its date (7, 14 or 30 days).	Ask your NCO for a new one.
“QR not valid” or “Link not valid”	It was revoked or copied incomplete.	Ask for a new QR; if you were revoked, talk to your NCO.
“Open it in your app”	The approval belongs to another phone or browser.	Open it in the app where you made the request, or use your personal link.
Sign-up does not finish	No internet or the form did not load.	Get online and scan the QR again.
Camera does not open	Camera permission blocked.	Allow the camera for the site in phone Settings, use “Choose a photo or screenshot” or scan with the Camera app.
I don't see the new features	Old offline copy.	More → Settings → Update app (needs internet).
My notes are gone	Daily auto-delete, “Clear all/Delete everything” or the browser cleared data.	Notes live only in that browser. Export a backup before the day ends.
Forgot the PIN	—	On the lock screen, “Delete everything” removes the PIN (and the data).
“Personal info detected”	You typed a number or detail that looks like an identifier.	Remove it from the note.
Official PDF fails	Browser support or low memory.	Use Print.
No Install button	Already installed or the browser does not allow it.	On iPhone use Safari → Share → Add to Home Screen.
Triage list is empty	It deletes itself after 12 hours.	Scan the QRs again.
Manual opens on page 1	Some viewers (iPhone) ignore the page jump.	Scroll to the page number shown next to the chapter.
Text too small	—	More → Text size A+.

14 Glossary

Term	Meaning
68W	Army combat medic (MOS).
ADTMC	Algorithm-Directed Troop Medical Care, MEDCOM Pam 40-7-21 (13 Nov 2019).
AEM	Advanced Emergency Medicine / advanced provider who sees “AEM Now”.
AMPLE / SAMPLE	Allergies, Medications, Past history, Last meal, Events (and Symptoms).
AOC / LOC / PTA	Alteration / loss of consciousness / post-traumatic amnesia.
CMT	Concussion Management Tool.
CoC	Chain of command.
DA 5181	Screening Note of Acute Medical Care.
DD 689	Individual Sick Slip.
e-Profile	Army system for medical profiles.
H2F	Holistic Health and Fitness (brigade classes and physical therapy).
LMP	Last menstrual period.
Light Duty	Duty with restrictions.
MACE 2	Military Acute Concussion Evaluation 2.
MCP	Minor-Care Protocol: treat here.
MRC	Medical Readiness Classification.
NSI	Neurobehavioral Symptom Inventory.
OPQRST	Onset, Provocation/Palliation, Quality, Region/Radiation, Severity, Time.
OTC / Rx	Over the counter / prescription.
PFB	Pseudofolliculitis barbae (razor bumps).
PII / PHI	Personally identifiable / protected health information.
PMS	Distal pulse, motor and sensation.
POC	Point of contact.
PN	Provider Now.

Term	Meaning
PRA	Progressive Return to Activity.
PWA	Progressive web app: installs from the browser and works offline.
Unit QR	Signed code that lets the medics of a unit sign up.
Quarters	Rest in quarters.
RTD	Return to Duty.
Service worker	Part of the PWA that keeps the offline copy.
SOAP-D	Subjective, Objective, Assessment, Plan, Disposition.
UCMJ Art. 115	Malingering.
VOMS	Vestibular/Ocular Motor Screening.

15 Under the hood: code and architecture

This part is for whoever maintains the app. Sick Call is a static site (HTML + CSS + JavaScript, no framework and no build step) published with GitHub Pages. There is no backend: everything runs in the browser.

Files

File	Contents
index.html	Structure: top bar, #main, the note (#aside), tabs and script loading in order.
style.css	Styles, themes (data-theme) and light/dark mode.
app.js	Logic (~3,200 lines): views, ADTMC engine, note builder, MACE, profiles, H2F, PFB, forms, settings, triage, updates. Defines APP_VER.
access.js	Access: ECDSA signatures, start screen, guest, unit QR, personal link, revocation.
intake.html	Soldier questionnaire (standalone).
data/protocols.js	The 86 Appendix B protocols as data.
data/meds.js · medinfo.js	Appendix C, pharmacy OTC list and “what is it for”.
data/plans.js · notetags.js	Suggested plans and note phrases.
data/howto.js	Step-by-step test cards.
data/intake.js	QR areas, questions, encoding and triage; Mendoza Clinic data.
data/pfb.js · h2f.js	PFB and H2F class data.
data/es.js	Spanish interface dictionary.
data/access.js	Root public keys (ROOT_PUBS), built-in revocations and enforcement date.
data/whatsnew.js	What's new window entries.
sw.js · manifest.json · version.json	Offline, install and version notice.
revoked.json	Revoked IDs list (downloaded on open).
vendor/	pdf-lib 1.17.1 (MIT), qrcode-generator 2.0.4 (MIT), jsQR 1.4.0 (Apache 2.0).
forms/DD689.pdf	DD 689 template (AUG 2017, public domain).

File	Contents
poster.html · demo.html	QR poster and demo.
tools/build_single.py	Builds dist/SICK_CALL_TOOL.html, one self-contained file.

ADTMC engine

Each protocol is declared with `P(id, category, title, {…})`: differential (`ddx`), red flags (`rf`), activity, treatment (`tx`), return to clinic (`rtc`), meds and the `flow` chart, a list of decision points `DP(items, yes, opts)`. A DP with `{d:"PN"}` leads to Provider Now if anything applies; `{a:…, out:[…]}` asks for an action with outcomes; `{tri:[…]}` routes to another protocol; `{min:3}` requires a minimum number of criteria (for example Centor).

```
P("B-8","b","Knee Pain",{pg:43,
 rf:["Abnormal PMS (pulse / motor / sensation)","Deformity","High-energy trauma"],
 flow:[
  DP(["Red flags present","Red / warm","Immediate swelling after trauma","No mechanism of
injury"],{d:"PN",a:"Immobilize the injured extremity before transport"}),
  DP(["Swelling","Decreased ROM","Previous knee injury"],{d:"AEM",a:"PT if available"})
 ], ...});
```

`adEval(p)` walks the flowchart with the medic's answers (`ad.rf`, `ad.dp`, `ad.out`), applies the global vital-signs rule and returns the final category. Categories live in `LV` (PN, AEM, APPT, SP, MCP) with a rank, so the highest wins. `renderNote()` builds the note from state (compact or SOAP-D) and `noteEntries` adds H2F, PFB and profile lines.

State and storage

Key (localStorage)	Contents
sc_state · sc_ad	Current case: template, history, protocol, answers.
sc_saved	Saved patients.
sc_settings	Settings (theme, AEM, folding, PIN, daily delete, what's new seen...).
sc_lang · sc_fs	Language and text size.
sc_iq	Triage list (12 h).
sc_dev	Random device ID.
sc_auth	Medic access: signed invite or approval, registration and personal-link ID.
sc_appr · sc_akey	Approver certificate and keys.
sc_root	Admin key (only on the owner's phone).

Key (localStorage)	Contents
sc_aplog	Log of approvals and QRs created.
sc_rev	Cached revocations.
sc_guest	Guest mode.

Serverless access (ECDSA P-256 signatures)

Every access is a `payload.signature` token in base64url, signed with WebCrypto (ECDSA P-256 / SHA-256) and verified against the root public keys in `data/access.js`. It travels as a QR or a link with a `#…=` fragment that never reaches a server.

Fragment	Type	Payload / who signs
#areq=	Request	{t:"req", r:medic|appr, d:device, u:unit, n:label, pub?} — unsigned.
#acert=	Approver certificate	{t:"cert", id, pub, u, iat} — signed by root.
#acc=	Approval	{t:"ok", id, d, u, iat, c:cert|null} — signed by an approver or root; bound to the device.
#ainv=	Unit QR	{t:"inv", id, u, exp, n, iat, c} — signed by an approver or root.
#apass=	Personal link	{t:"pass", id, k:inv|acc, a:inner token, reg} — the inner token is verified again.
#aroot=	Admin key	Installs the root private key on the owner's phone. Never goes to the repository.
#iq=	Patient QR	Encoded questionnaire answers (unsigned).

`AC.role()` decides the role on every start: root, approver, medic or none. It checks revocations by device ID, personal link, invite and approval. `acRevSync()` downloads `revoked.json` on open and when the app comes back to the screen. Unit-QR sign-up uses an embedded Tally form and grants access on the `Tally.FormSubmitted` event from tally.so.

WARNING The root private key gives full control of access. Never commit it to the repository or share it.

Offline, versions and updates

`sw.js` caches every file in the `sickcall-vNN` cache and serves network-first: online it gets the newest files, offline it uses the copy. On open and on return, `app.js` compares `version.json` with `APP_VER`; if newer, it saves and reloads once. The What's new window shows once per version (`settings.wnSeen`).

Languages

The interface uses `L(en, es)` in newer code and the `data/es.js` dictionary for older text; a `MutationObserver` translates anything added to the screen. The note is always generated in English.

DD 689 PDF

`slipPdf()` loads `forms/DD689.pdf` with pdf-lib, fills the fields, flattens them, removes annotations, adds the footer stamp and metadata, and downloads the file. All on the phone.

How to release a version

1. Work on a new branch and open a pull request; the owner reviews and squash-merges.
2. Bump the version in three places together: `APP_VER` in app.js, the cache name in sw.js and `version.json`.
3. Add an entry at the top of `data/whatsnew.js` (1–3 items, EN and ES) and update `CHANGELOG.md` in EN and ES.
4. When merged to main, GitHub Pages publishes; phones update by themselves on open.

Testing is done with Playwright (Chromium) at phone size, serving the repository with `python3 -m http.server`. Clinical changes must cite the ADTMC page.

Known limits

- ADTMC flowchart F-2 (headache) is missing from the source PDF; it was built from the printed narrative.
- The 2019 ADTMC lists ranitidine (withdrawn from the US market); the app points to famotidine / local formulary.
- Appendices E–G are included only as condensed summaries; the pamphlet is authoritative.
- The CMT symptom-cluster tables and the PRA flowchart graphic are not reproduced; the rules, stages and NSI are.
- Spanish interface text was machine-translated and not clinically reviewed.

Appendix A ADTMC protocol list

Code	Protocol
A · Ear, nose and throat	
A-1	Sore Throat / Hoarseness
A-2	Ear Pain / Drainage / Trauma
A-3	Cold Symptoms / Allergies / Cough
A-4	Ringing in the Ears / Hearing Problem
A-5	Nosebleed / Nose Trauma
B · Musculoskeletal	
B-1	Back Pain
B-2	Neck Pain
B-3	Shoulder Pain
B-4	Elbow Pain
B-5	Wrist Pain
B-6	Hand Pain
B-7	Hip Pain
B-8	Knee Pain
B-9	Ankle Pain
B-10	Foot Pain
B-11	Extremity, Non-Joint Pain
C · Gastrointestinal	
C-1	Nausea / Vomiting
C-2	Diarrhea
C-3	Abdominal and Flank Pain
C-4	Rectal Pain / Itching / Bleeding
C-5	Constipation
C-6	Difficulty Swallowing (Dysphagia)
C-7	Heartburn
D · Cardio-respiratory	
D-1	Shortness of Breath
D-2	Chest Pain
E · Genitourinary	

Code	Protocol
E-1	Painful / Frequent Urination
E-2	Groin / Testicular Pain or Urethral Discharge
E-3	Sexually Transmitted Infection (STI)
E-4	Problems with Voiding
F · Neurological / behavioral health	
F-1	Dizziness / Faintness / Blackout
F-2	Headache
F-3	Numbness / Tingling / Paralysis / Weakness
F-4	Drowsiness / Confusion
F-5	Depression / Nervousness / Anxiety / Tension
F-6	Mild Traumatic Brain Injury (Concussion)
G · General	
G-1	Fatigue
G-2	Fever / Chills
H · Eyes	
H-1	Eye Pain / Redness / Discharge / Itching / Injury
H-2	Eyelid Problem
H-3	Decreased Vision / Seeing Spots / Request for Glasses
H-4	Seeing Double (Diplopia)
I · Women's health	
I-1	Breast Problems
I-2	Suspects Pregnancy
I-3	Menstrual Problems / Vaginal Bleeding
I-4	Vaginal Discharge / Itching / Irritation / Pain
I-5	Request for Pap / Routine Pelvic Exam
I-6	Request for Information on Contraception
J · Skin	
J-1	Unknown Cause of Skin Disorder / Complaint
J-2	Acne
J-3	Shaving Problem — Pseudofolliculitis Barbae
J-4	Dandruff (Scaling of the Scalp)
J-5	Hair Loss
J-6	Athlete's Foot (Tinea Pedis)

Code	Protocol
J-7	Jock Itch (Tinea Cruris)
J-8	Tinea Versicolor (Scaling, Depigmented Spots on Chest, Back, Upper Arms)
J-9	Boils / Abscess
J-10	Fever Blister / Cold Sore
J-11	Abrasion / Laceration
J-12	Suture Removal
J-13	Drug Rash / Contact Dermatitis
J-14	Burns / Sunburn
J-15	Friction Blisters
J-16	Corns on Feet
J-17	Cutaneous Warts
J-18	Ingrown Toenail
K · Environmental	
K-1	Heat Injury / Hyperthermia (Cramps, Exhaustion, Heatstroke)
K-2	Hypothermia
K-3	Immersion Foot (Non-Freezing Cold Injury)
K-4	Chapped Skin / Windburn
K-5	Frostbite
K-6	Crabs / Lice (Pediculosis)
K-7	Insect Bites (not crabs/lice)
L · Miscellaneous	
L-1	Exposed to Hepatitis B / C or HIV
L-2	Dental Problems
L-3	Sores in the Mouth
L-4	Prescription Refill
L-5	Requests a Vasectomy
L-6	Needs an Immunization
L-7	Lymph Node Enlargement
L-8	Blood Pressure Check
L-9	Medical Screening for Overseas PCS
L-10	Weight Reduction
L-11	Complaint Not on the List
L-12	Request for Nonprescription or Traveling Medications

Code	Protocol
M · Return	
M-1	No Signs of Improvement (Not Getting Better)
M-2	Return Requested by Provider

Appendix B Version history

Summary of the What's new window. Full detail is in `CHANGELOG.md`.

Version	What's new
v37	**Operator's manual in the app** — More → Operator's manual: the full TM-style guide (EN/ES PDF). Tap a chapter to open it on that page; it also suggests the chapter for the screen you came from. Works offline after you open it once.
v36	**Mendoza Clinic sick call rules** — More → Mendoza Clinic (hours, DD 689, phones, what sick call is for). The patient QR now asks if the problem is new and if they only want documentation; injuries older than 48 h → H2F, documentation → appointment.
v35	**H2F class referral** — on MSK protocols (B-) and in Profiles: pick the H2F class(es), times per week and weeks; the note line writes itself and you can print the H2F slip prefilled. Unit-level profiles now go up to 14 days; PT and longer profiles → clinic (Mendoza).
v34	**Your personal access link** — More → Access → "My personal access link". Save it (email, notes, WhatsApp to yourself). If you open the app in another browser or a new phone, open the link and you're back in — no sign-up.
v29	**Unit QR — easier medic access** — NCOs create one QR for the unit (More → Access → Create unit QR). New medics scan it, fill rank, name, MOS, unit and POC once, and they're in. Already-approved phones don't need to do anything.
v28	**Guest MACE = quick screen only** — guests answer 1D and 2A–2D and get Positive / Negative with "go to a medic". The full exam, scoring, PRA and notes stay for approved medics.
v27	**MACE result stays on top** — the screening card stays pinned while you scroll and answer.
v26	**Cleaner start screen + guest mode** — anyone can "Continue as a guest" with reference tools only (Meds, MACE, Assessments). Protocols, profiles and notes stay for approved medics. **Cleaner MACE 2** — live result at the top and folded sections (screening, cognitive, neuro, PRA); NSI on one line per symptom.
v25	**What's new window** — after each update you'll see this card with the new features.
v24	**Shaving profile (PFB)** — on ADTMC J-3 and More → Profiles: pick Mild / Moderate / Severe and get the phase, plan and e-Profile wording to copy. Soldiers can also check in for it on the QR questionnaire.
v23	**"What is it for" on every med** — tap ⓘ next to a med: what it does, what it's best for and when to pick another.
v22	**Meds by groups** — the Meds tab is organized by use with an All / OTC / Rx filter, plus the clinic **Pharmacy Self-Care OTC list** (Aug 2025).
v21	**Install button** — add the app to your Home Screen from the button at the top (Android and iPhone).

Appendix C Credits, license and contact

- Author and maintainer: bubymed (github.com/bubymed).
- License: source-available, all rights reserved. You may view the code and use the tool personally; copying, modifying or redistributing it requires written permission from the author (see LICENSE).
- Third party: pdf-lib 1.17.1 (MIT), qrcode-generator 2.0.4 (MIT), jsQR 1.4.0 (Apache 2.0), DD Form 689 (public domain). Details in THIRD_PARTY.md.
- Clinical source: MEDCOM Pam 40-7-21, Algorithm-Directed Troop Medical Care (13 Nov 2019).
- Suggestions: issues or pull requests in the repository, or the feedback survey (More → Feedback). Clinical changes must cite the ADTMC page.